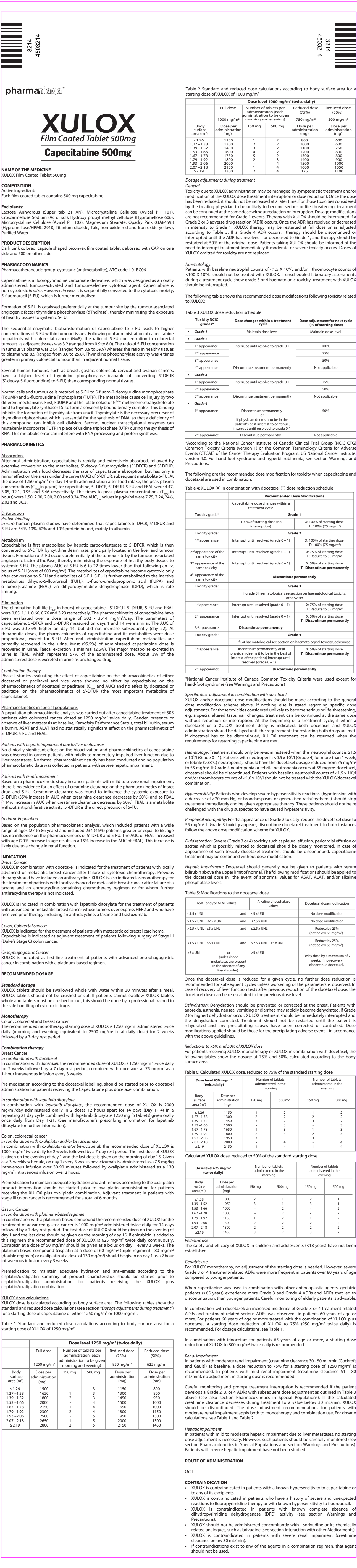




Capecitabine Tablets 500 mg - KLD Leaflet
Size : 150 x 820 mm Folding size: 150x40 mm

35 mm

WARNINGS AND PRECAUTIONS

Warning

Diarrhea

Capecitabine can induce diarrhea, which can sometimes be severe. Patients with severe diarrhea should be carefully monitored and given fluid and electrolyte replacement if they become dehydrated. Standard antidiarrhoeal treatments (e.g. loperamide) should be initiated, as medically appropriate, as early as possible. Dose reduction should be applied as necessary.

Dehydration

Dehydration should be prevented or corrected at the onset. Patients with anorexia, asthenia, nausea, vomiting or diarrhea may rapidly become dehydrated. Dehydration may cause acute renal failure, especially in patients with pre-existing compromised renal function or when capecitabine is given concomitantly with known nephrotoxic agents. Fatal outcome of renal failure has been reported in these situations. If grade 2 (or higher) dehydration occurs, capecitabine treatment should be immediately interrupted and the dehydration corrected. Treatment should not be restarted until the patient is rehydrated and any precipitating causes have been corrected or controlled. Dose modifications should be applied for the precipitating adverse event as necessary (see section Recommended Dosage).

Dihydropyrimidine dehydrogenase (DPD) deficiency

Rarely, unexpected, severe toxicity (e.g. stomatitis, diarrhea, mucosal inflammation, neutropenia and neurotoxicity) associated with 5-FU has been attributed to a deficiency of DPD activity, an enzyme involved in fluorouracil degradation.

Patients with certain homozygous or certain compound heterozygous mutations in the DPD gene locus that cause complete or near complete absence of DPD enzymatic activity are at the highest risk for severe life-threatening or fatal adverse reactions caused by fluorouracil. These patients should not be treated with capecitabine. No dose has been proven safe for patients with complete absence of DPD activity (see section Contraindications).

Patients with certain heterozygous DPDV variants (e.g. DPDV*2A variants) that may cause partial DPD deficiency have been shown to have increased risk of severe toxicity when treated with capecitabine.

For patients with partial DPD deficiency the benefits of capecitabine are considered to outweigh the risks (taking into account the suitability of an alternative non-fluoropyrimidine chemotherapeutic regimen), these patients must be treated with extreme caution, initially with a substantial dose reduction and frequent monitoring and dose adjustment according to toxicity. Testing for DPD deficiency should be considered based on the local availability and current guidelines.

In patients with unrecognized DPD deficiency treated with capecitabine as well as patients who test negative for specific DPDV variations, life-threatening toxicities manifesting as acute, in the event of grade 2-4 acute toxicity, treatment must be discontinued immediately. Permanent discontinuation should be considered based on clinical assessment of the onset, duration and severity of the observed toxicities (see section Symptoms and Treatment of Overdose).

Precautions

The spectrum of cardiotoxicity observed with capecitabine is similar to that of other fluorinated pyrimidines. This includes myocardial infarction, angina, dysrhythmias, cardiac arrest, cardiac failure and electrocardiographic changes. These ADRs may be more common in patients with a prior history of coronary artery disease.

Capecitabine can induce severe skin reactions such as Stevens-Johnson syndrome and Toxic Epidermal Necrolysis (TEN). Capecitabine should be permanently discontinued in patients who experience a severe skin reaction possibly attributed to capecitabine treatment.

Capecitabine can induce hand-foot syndrome (palmar-plantar erythrodysesthesia or chemotherapy induced acral erythema) which is a cutaneous toxicity. Persistent or severe hand-foot syndrome (grade 2 and above) can eventually lead to loss of fingerprints, which could impact patient identification. For patients receiving capecitabine monotherapy in the metastatic setting, the median time to onset of 79 days, range from 11 to 360 days), with a severity range of Grades 1 to 3.

Grade 1 hand-foot syndrome is defined by numbness, dysesthesia/paresthesia, tingling, or erythema of the hands and/or feet and/or discomfort which does not disrupt normal activities. Grade 2 is defined as painful erythema and swelling of the hands and/or feet and/or discomfort affecting the patient's activities of daily living. Grade 3 is defined as moist desquamation, ulceration, blistering or severe pain of the hands and/or feet and/or severe discomfort that causes the patient to be unable to work or perform activities of daily living. If Grade 2 or 3 hand-foot syndrome occurs, administration of capecitabine should be interrupted until the event resolves or decreases in intensity to Grade 1. Following Grade 3 hand-foot syndrome, subsequent doses of capecitabine should be decreased (see section Recommended Dosage). When capecitabine and cisplatin are used in combination, use of Vitamin B6 (pyridoxine) is not advised for the symptomatic or secondary prophylactic treatment of hand-foot syndrome, because of published reports that it may decrease the efficacy of cisplatin. There is some evidence that dexpanthenol is effective for hand-foot syndrome prophylaxis in patients treated with capecitabine.

Capecitabine can induce hyperbilirubinemia. Administration of capecitabine should be interrupted if treatment-related elevations in bilirubin of > 3.0 x ULN or treatment-related elevations in hepatic aminotransferases (ALT, AST) of > 2.5 x ULN occur. Treatment may be resumed when bilirubin decreases to ≤3.0 x ULN or hepatic aminotransferases decrease to ≤2.5 x ULN.

Care should be exercised when capecitabine is co-administered with drugs, which are metabolized by cytochrome P450 2C9 such as for example warfarin or phenytoin. Patients receiving concomitant capecitabine and oral coumarin-derivative anticoagulant therapy should have their anticoagulant response (INR or prothrombin time) monitored closely and the anticoagulant dose adjusted accordingly. Patients taking phenytoin concomitantly with capecitabine should be regularly monitored for increased phenytoin plasma concentrations (see section Interactions with other Medicaments).

Lactose

Capecitabine contains lactose. Patients with rare hereditary problems of galactose intolerance, total lactase deficiency or glucose-galactose malabsorption should not take this medicine.

Sodium

This medicine contains less than 1mmol sodium (23mg) per, that is to say essentially 'sodium-free'.

INTERACTION WITH OTHER MEDICAMENTS

Cytochrome P-450 2C9 substrates

No formal drug-drug interaction studies with capecitabine and other drugs known to be metabolized by the cytochrome P450 2C9 isoenzyme have been conducted. Care should be exercised when capecitabine is co-administered with these drugs.

Coumarin anticoagulants

Altered coagulation parameters and/or bleeding have been reported in patients taking capecitabine concomitantly with coumarin-derivative anticoagulants such as warfarin and phenprocoumon. These events occurred within several days and up to several months after initiating capecitabine therapy and, in a few cases, within one month after stopping capecitabine. In a clinical interaction study, after a single 20 mg dose of warfarin, capecitabine treatment increased the AUC of S-warfarin by 57% with a 91% increase in INR value. These results suggest an interaction, probably due to an inhibition of the cytochrome P450 2C9 isoenzyme system by capecitabine. Patients taking coumarin-derivative anticoagulants concomitantly with capecitabine should be monitored regularly for alterations in their coagulation parameters (PT or INR) and the anticoagulant dose adjusted accordingly (see Warning and Precautions).

Phenytoin

Increased phenytoin plasma concentrations have been reported during concomitant use of capecitabine with phenytoin. Formal drug-drug interaction studies with phenytoin have not been conducted, but the mechanism of interaction is presumed to be inhibition of the CYP2C9 isoenzyme system by capecitabine (see Coumarin anticoagulants). Patients taking phenytoin concomitantly with capecitabine should be regularly monitored for increased phenytoin plasma concentrations.

Antacid

The effect of an aluminium hydroxide and magnesium hydroxide-containing antacid on the pharmacokinetics of capecitabine was investigated in cancer patients. There was a small increase in plasma concentrations of capecitabine and one metabolite (5'-DFCR); there was no effect on the 3 major metabolites (5'-DFUR, 5-FU and FBAL).

Leucovorin (folinic acid)

The effect of leucovorin on the pharmacokinetics of capecitabine was investigated in cancer patients. Leucovorin has no effect on the pharmacokinetics of capecitabine and its metabolites. However, leucovorin has an effect on the pharmacodynamics of capecitabine and its toxicity may be enhanced by leucovorin.

Sorivudine and analogues

A clinically significant drug-drug interaction between sorivudine and 5-FU, resulting from the inhibition of dihydropyrimidine dehydrogenase by sorivudine, has been described in the literature. This interaction, which leads to increased fluoropyrimidine toxicity, is potentially fatal. Therefore, capecitabine should not be administered concomitantly with sorivudine or its chemically related analogues, such as brivudine (see section Contraindications). There must be at least a 4-week waiting period between the end of treatment with sorivudine or its chemically related analogues such as brivudine and start of capecitabine therapy.

Oxaliplatin

No clinically significant differences in exposure to capecitabine or its metabolites, free platinum or total platinum occurred when capecitabine and oxaliplatin were administered in combination, with or without bevacizumab.

Bevacizumab

There was no clinically significant effect of bevacizumab on the pharmacokinetic parameters of capecitabine or its metabolites.

Drug-food interaction

In all clinical trials, patients were instructed to administer capecitabine within 30 minutes after a meal. Since current safety and efficacy data are based upon administration with food, it is recommended that capecitabine be administered with food.

FERTILITY, PREGNANCY AND LACTATION

Use In Special Populations

Female and Males of Reproductive Potential

Fertility

Based on evidence from animal studies, capecitabine may impair fertility in females and males of reproductive potential.

Contraception

Females

Women of childbearing potential should be advised to avoid becoming pregnant while receiving treatment with capecitabine. An effective method of contraception should be used during treatment and for 6 months after the last dose of capecitabine. If the patient becomes pregnant while receiving capecitabine, the potential hazard to the fetus must be explained.

Males

Based on genetic toxicity findings, male patients with female partners of reproductive potential should use effective contraception during treatment and for 3 months following the last dose of capecitabine.

Pregnancy

There are no studies in pregnant women using capecitabine; however, based on the pharmacological and toxicological properties of capecitabine, it can be assumed that capecitabine may cause fetal harm if administered to pregnant women. In reproductive toxicity studies in animals, capecitabine administration caused embryolethality and teratogenicity. These findings are expected effects of fluoropyrimidine derivatives. Capecitabine should be considered a potential human teratogen. Capecitabine should not be used during pregnancy. If capecitabine is used during pregnancy or if the patient becomes pregnant while receiving this drug, the patient must be apprised of the potential hazard to the fetus.

Lactation

It is not known whether capecitabine is excreted in human breast milk. No studies have been conducted to assess the impact of capecitabine on milk production or its presence in human breast milk. In a study of single oral administration of capecitabine in lactating mice, a significant amount of capecitabine and its metabolites were found in the milk. As the potential for harm to the nursing infant is unknown, breastfeeding should be discontinued during treatment with capecitabine and for 2 weeks after the final dose.

ADVERSE EFFECTS

Clinical Trials

Adverse drug reactions (ADRs) considered by the investigator to be possibly, probably, or remotely related to the administration of capecitabine have been obtained from clinical studies conducted with capecitabine monotherapy (in adjuvant therapy of colon cancer, in metastatic colorectal cancer and metastatic breast cancer), and clinical studies conducted with capecitabine in combination with different chemotherapy regimens for multiple indications. ADRs are added to the appropriate category in the tables below according to the highest incidence from the pooled analysis of seven clinical trials. Within each frequency grouping ADRs are listed in descending order of seriousness. Frequencies are defined as very common ≥ 1/10, common ≥ 1/100 to < 1/10 and uncommon ≥ 1/1000 to < 1/100.

Capecitabine monotherapy

Safety data of capecitabine monotherapy were reported for patients who received adjuvant treatment for colon cancer and for patients who received treatment for metastatic breast cancer or metastatic colorectal cancer. The safety information includes data from a phase III trial in adjuvant colon cancer (995 patients treated with capecitabine and 974 treated with i.v. 5-FU/LV) and from 4 phase II trials in female patients with breast cancer (N=319) and 3 trials (1 phase II and 2 phase III trials) in male and female patients with colorectal cancer (N=630). The safety profile of capecitabine monotherapy is comparable in patients who received adjuvant treatment for colon cancer and in those who received treatment for metastatic breast cancer or metastatic colorectal cancer. The intensity of ADRs was graded according to the toxicity categories of the NCIC CTC Grading System.

Table 7: Summary of ADRs reported in ≥5% of patients treated with capecitabine monotherapy

Body System ADR	Very Common (10%)	Common (5% to <10%)
Metabolism and nutrition disorders	Anorexia (G3/4: 1%)	Dehydration (G3/4: 3%) Appetite decreased (G3/4:<1%)
Nervous system disorders		Paresthesia, Dyspnoea (G3/4:<1%), Headache (G3/4:<1%), Dizziness (excl. vertigo) (G3/4:<1%)
Eye disorders		Lacrimation increased Conjunctivitis (G3/4:<1%)
Gastrointestinal disorders	Diarrhea (G3/4: 13%) Vomiting (G3/4: 4%) Nausea (G3/4: 4%) Stomatitis (all*) (G3/4: 4%) Abdominal pain (G3/4: 3%)	Constipation (G3/4:<1%), Abdominal pain upper (G3/4:<1%), Dyspepsia (G3/4:<1%)
Hepatobiliary disorders		Hyperbilirubinemia (G3/4:1%)
Skin and subcutaneous tissue disorders	Palmar-plantar erythrodysesthesia syndrome ** (G3/4: 17%), Dermatitis (G3/4:<1%)	
General disorders and administration site conditions	Fatigue (G3/4: 3%), Lethargy (G3/4:<1%)	Pyrexia (G3/4:<1%), Weakness (G3/4:<1%), Asthenia (G3/4:<1%)

* Stomatitis, mucosal inflammation, mucosal ulceration, mouth ulceration

** Based on the post-marketing experience, persistent or severe palmar-plantar erythrodysesthesia syndrome can eventually lead to loss of fingerprints (see section Warnings and Precautions).

Skin fissures were reported to be at least remotely related to capecitabine in less than 2% of the patients in seven completed clinical trials (N= 949).

The following ADRs represent known toxicities with fluoropyrimidine therapy and were reported to be at least remotely related to capecitabine in less than 5% of patients in seven completed clinical trials (N = 949):

- Gastrointestinal disorder: dry mouth, flatulence, ADRs related to inflammation/ulceration of mucous membranes such as esophagitis, gastritis, duodenitis, colitis, gastrointestinal hemorrhage.
- Cardiac disorders: edema lower limb, cardiac chest pain including angina, cardiomyopathy, myocardial ischemia/infarction, cardiac failure, sudden death, tachycardia, atrial arrhythmias including atrial fibrillation, and ventricular extrasystoles.
- Nervous system disorders: taste disturbance, insomnia, confusion, encephalopathy, and cerebellar signs such as ataxia, dysarthria, impaired balance, abnormal coordination.
- Infections and infestations: ADRs related to bone marrow depression, immune system compromise, and/or disruption of mucous membranes such as local and fatal systemic infections including bacterial, viral, fungal etiologies) and sepsis.
- Blood and lymphatic system disorders: anemia, bone marrow depression/ pancytopenia.
- Skin and subcutaneous tissue disorders: pruritus, localized exfoliation skin hyperpigmentation, nail disorders, photosensitivity reactions, radiation recall syndrome.
- General disorders and administration site conditions: pain in limb, chest pain (non-cardiac).
- Eye: eye irritation.
- Respiratory: dyspnoea, cough.
- Musculoskeletal: back pain, myalgia, arthralgia.
- Psychiatric disorders: depression.
- Hepatic failure and cholestatic hepatitis have been reported during clinical trials and post-marketing exposure. A causal relationship with capecitabine treatment has not been established.

Capecitabine in combination therapy

Table 8 lists ADRs associated with the use of capecitabine in combination therapy with different chemotherapy regimens in multiple indications and occurred in addition to those seen with monotherapy and/or at a higher frequency grouping. The safety profile was similar across all indications and combination regimens. These reactions occurred in ≥5% of patients treated with capecitabine in combination with other chemotherapies. Adverse drug reactions are added to the appropriate category in the table below according to the highest incidence seen in any of the major clinical trials. Some of the adverse reactions are reactions commonly seen with chemotherapy (e.g. peripheral sensory neuropathy with docetaxel or oxaliplatin), or with bevacizumab (e.g. hypertension); however, an exacerbation by capecitabine therapy cannot be excluded.

Table 8: Very common and common ADRs for capecitabine in combination with different chemotherapies in addition to those seen for capecitabine monotherapy.

Body System Adverse Event	Very Common ≥10%	Common ≥5% to <10%
Infections and Infestations		Infection + Oral candidiasis
Blood and lymphatic system disorders	Neutropenia +, Leukopenia +, Fibrinogenopenia +, Thrombocytopenia +, Anemia +	
Metabolism and nutrition disorders	Appetite decreased	Hypokalemia, Weight Decreased
Psychiatric disorders		Insomnia
Nervous system disorders	Neuropathy peripheral, Peripheral sensory neuropathy, Neuropathy, Paresthesia, Dyspnoea, Dysaesthesia, Headache	Hypoesthesia
Eye disorders	Lacrimation increased	
Vascular Disorders	Thrombosis/embolism, Hypertension, Lower limb oedema	
Respiratory, thoracic and mediastinal disorders	Dysaesthesia pharynx, Sore throat	Epistaxis, Dysphonia, Rhinorrhoea, Dyspnoea
Gastrointestinal disorders	Constipation, Dyspepsia	Dry mouth
Skin and subcutaneous tissue disorders	Alpecia, Nail disorder	
Musculoskeletal and connective tissue disorders	Arthralgia, Myalgia, Pain in extremity	Pain in jaw, Back Pain
General disorders and administration site conditions	Pyrexia, Asthenia, Weakness, Temperature intolerance	Fever + Pain

Frequencies based on all grades except those denoted with *, which are based on G3/4 ADRs only.

Hypersensitivity reactions (2%) and cardiac ischaemia/infarction (3%) have been reported commonly for capecitabine in combination with other chemotherapy but in less than 5% of patients.

Rare or uncommon ADRs reported for capecitabine in combination with other chemotherapy are consistent with the ADRs reported for capecitabine monotherapy or the combination product monotherapy (see prescribing information for the combination product).

Laboratory Abnormalities

The following table displays laboratory abnormalities observed in 995 patients (adjuvant breast cancer and 949 patients (metastatic breast and colorectal cancer), regardless of relationship to treatment with capecitabine.

Table 9: Laboratory abnormalities: Capecitabine monotherapy in adjuvant colon cancer and in metastatic breast and colorectal cancer

Parameter*	Capecitabine 1250 mg/m ² twice daily intermittent Patients with Grade 3 / 4 abnormality (%)
Increased ALAT (SGPT)	1.6
Increased ASAT (SGOT)	1.1
Increased alkaline phosphatase	3.5
Increased calcium	1.1
Decreased calcium	2.3
Decreased granulocytes	0.3
Decreased hemoglobin	3.1
Decreased lymphocytes	44.4
Decreased neutrophils	3.6
Decreased neutrophils/granulocytes	2.4
Decreased platelets	2.0
Decreased potassium	0.3
Increased serum creatinine	0.5
Decreased sodium	0.4
Increased bilirubin	20.0
Hyperycemia	4.4

* Laboratory abnormalities were graded according to the categories of the NCIC CTC Grading System

Table 10: Adverse Drug Reactions from Postmarketing Experience

System Organ Class (SOC)	ADRs(s)	Frequency
Renal and urinary disorders	Acute renal failure secondary to dehydration including fatal outcome	Rare
Nervous system disorders	Toxic leukoencephalopathy	Unknown
Hepatobiliary disorders	Hepatic failure, Cholestatic hepatitis	Very rare
Metabolism and nutrition disorders	Hypertriglyceridemia	Unknown
Skin and subcutaneous tissue disorders	Cutaneous lupus erythematosus, Severe skin reactions such as Stevens-Johnson Syndrome and Toxic Epidermal Necrolysis (TEN)	Very rare
Eye disorders	Lacrimal duct stenosis NOS, Corneal disorders including keratitis	Very rare

* This subtype of hypersensitivity reaction was reported in the postmarketing setting.

Exposure to crushed or cut capecitabine tablets:

In the instance of exposure to crushed or cut capecitabine tablets, the following ADRs have been reported: eye irritation, eye swelling, skin rash, headache, paresthesia, diarrhea, nausea, gastric irritation, and vomiting.

Special populations

Paediatric patients

The safety and efficacy of capecitabine in paediatric patients below the age of 18 have not been established.

Geriatric patients

Among patients with colorectal cancer aged 60-79 years receiving capecitabine monotherapy in the metastatic setting, the incidence of gastrointestinal toxicity was similar to that in the overall population. In patients aged 80 years or older, a larger percentage experienced reversible Grade 3 or 4 gastrointestinal ADRs, such as diarrhea, nausea and vomiting (see section Recommended Dosage). When capecitabine was used in combination with other agents geriatric patients (≥ 65 years) experienced more Grade 3 and Grade 4 ADRs and ADRs that led to discontinuation than younger patients. An analysis of safety data in patients equal to or greater than 60 years of age treated with capecitabine plus docetaxel combination therapy showed an increase in the incidence of treatment-related Grade 3 and 4 ADRs, treatment-related serious ADRs and early withdrawals from treatment due to ADRs compared to patients less than 60 years of age.

Renal impairment

Physicians should exercise caution when capecitabine is administered to patients with impaired renal function. As seen with 5-FU the incidence of treatment-related Grade 3 or 4 ADRs was higher in patients with moderate renal impairment (creatinine clearance 30-50 mL/min) (see section Recommended Dosage).

Hepatic Impairment

Patients with hepatic impairment should be carefully monitored when Capecitabine is administered. The effect of hepatic impairment not due to liver metastases or severe hepatic impairment on the disposition of capecitabine is not known (see section Pharmacokinetics Recommended Dosage).

SYMPTOMS AND TREATMENT OF OVERDOSE

The manifestations of acute overdose include nausea, vomiting, diarrhea, mucositis, gastrointestinal irritation and bleeding, and bone marrow depression. Medical management of overdose should include customary therapeutic and supportive medical interventions aimed at correcting the presenting clinical manifestations and preventing their possible complications.

EFFECT ON ABILITY TO DRIVE AND USE MACHINE

Capecitabine has moderate influence on the ability to drive and use machines. Patients should be advised to use caution when driving or using machines if they experience ADRs such as dizziness, fatigue, and/or nausea during treatment with capecitabine (see section Adverse Effects).

STORAGE CONDITIONS

Store below 30°C.

SHELF LIFE

24 months

DOSSAGE FORMS AND PACKAGING AVAILABLE

Pack of 120 tablets (12 blisters x 10 tablets)

MANUFACTURER

Shilpa Medicare Limited
Unit-4, Pharmaceutical Formulations SEZ, Plot No. S-20
to S-26, Pharma SEZ, TSIC,Green Industrial Park,
Polepally (V), Jadcherla (M),Mahabubnagar (Dt),
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PRODUCT REGISTRATION HOLDER

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DATE OF REVISION

11 Feb 2026